

Tetanus Vaccine Booster Standing Order Template

Issue date:	August 2024	Review date:	August 2025
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This standing order is to apply until it is either replaced by a new standing order covering the same subject matter or cancelled in writing by the issuer.

Standing Order Name	Tdap (Boosterix)
Rationale	Prevention of tetanus following injury
Scope (condition and patient group)	Children ≥ 7 years and adults with tetanus prone wounds, who have previously had at least 3 doses of a vaccine for primary immunisation for diphtheria and tetanus
Red Flags	N/A
Assessment	Review wound for the following that indicate an increased risk of tetanus infection: • Contaminated • Infected • Penetrating • More than 6 hours old • Tissue damage
Indication	Patients presenting with a tetanus prone wound and have not had a tetanus booster in the last 5 years.
Medicine	Tdap vaccine (trade names Boostrix or Adacel)
Assessment	Check whether a primary course of a pertussis containing vaccine been administered Check when the last dose was administered Check whether the person has any contraindications to the vaccine
Dosage instructions	One 0.5mL pre-filled syringe
Route of administration	IM Deltoid
Quantity to be given	One dose
Precautions	See Immunisation Handbook for pre-vaccination checklists and precautions https://www.tewhaturora.govt.nz/for-health-professionals/clinical-guidance/immunisation-handbook/21-tetanus/
Additional information	Small risk of anaphylaxis, therefore patient should wait for 20 minutes post injection. Verbal and written post vaccination (HE2505) information should be given The Authorised Vaccinator is responsible for all documentation related to the vaccination event.
Follow-up	Any adverse events related to this vaccine must be reported to CARM
Countersigning and auditing	Countersigning is not required. Audit: 50% monthly of administration and/ or supply records if there are 20 or fewer in total. 20-30% of administration and/ or supply records if they are in the range of 21-100.
Competency/training requirements	All healthcare professionals working under this standing order must be signed off as competent to do so by the issuer and have had specific training in this standing order. Healthcare professionals must have completed approved vaccinator training and ongoing two-year updates.
Supporting documentation	HealthPathways at www.healthpathways.org.nz Best Practice Journal at www.bpac.org.nz New Zealand Formulary at www.nzf.org.nz Individual medicine data sheets at www.medsafe.govt.nz Standing Order Guidelines, Ministry of Health, https://www.health.govt.nz/publication/standing-order-guidelines Immunisation Handbook www.immune.org.nz and https://www.immune.org.nz/factsheets/guidelines-for-the-management-of-tetanus-prone-wounds Medsafe https://www.medsafe.govt.nz/profs/PUArticles/ADRreport.htm

Definition of terms used in standing order	Tetanus prone wounds- All wounds that may be contaminated, infected, penetrating, more than 6 hours old and/or with extensive tissue damage. Non tetanus prone wounds- Clean, minor wounds that are less than 6 hours old, non-penetrating and with negligible tissue damage.
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Medical Centre or Clinic:	
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Signed by issuers

Name: _____

Signature: _____

Title: _____

Date: _____

Name: _____

Signature: _____

Title: _____

Date: _____

Healthcare professionals operating under Tetanus Vaccine Booster Standing Order
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Only registered healthcare professionals working within the above medical centre or clinic are authorised to administer medication under this standing order.

We the undersigned agree that we have read, understood and will comply with this standing order and all associated documents.

Name: _____ Signature: _____ Date: _____

Name: _____ Signature: _____ Date: _____

Name: _____ Signature: _____ Date: _____

Name: _____ Signature: _____ Date: _____

Name: _____ Signature: _____ Date: _____

Name: _____ Signature: _____ Date: _____

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